



Speech Pathology – Inpatient Management Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service	Allied Health	Speech Pathology

2. Purpose

To define eligibility criteria, referral processes, prioritisation and discharge of Speech Pathology (SP) inpatient services.

The SP department provides clinical services to adult (over 18) inpatients with swallowing, speech, language, voice and fluency disorders.

3. Procedure

3.1 Referrals

The SP department accepts referrals from medical, allied health and nursing staff. New referrals are accessed differently on each ward – as below:

3.1.1 MAU/Canning

- Via electronic Patient Journey Board (PJB) in allied health office/nurses' station, or
- Via pager numbers 159 or 219, or
- Verbally via extension 2395, or
- Verbally to SP directly in person

3.1.2 Carl Streich Rehab/RAC

- Referrals received via ward diary -located on front reception desk, or
- Via 8:30 morning meeting in handover room, or
- Via page number 260, or
- Verbally via extension 2367, or
- Verbally to SP directly in person

3.1.3 ED/ICU/Colyer/Mental Health*

- Via pager 602 or extension 2219, or
- Verbally to SP directly in person, or
- AHS.SpeechPathology@health.wa.gov.au
- *Mental Health (MH) – SP not funded to provide service however referrals will be considered according to priority and costed to MH.

3.2 Prioritisation and frequency of Services

- See Prioritisation Guidelines [Appendix 1](#) for full details.

3.3 Documentation

- The managing SP must document all service events in the integrated progress notes in accordance with [documentation standards](#)
- Consent must be obtained and mandatory patient identification descriptors (3 point ID check) according to [EMHS Patient Identification and Procedure Matching Policy](#).
- SP must update each patient's iSOFT entry daily according to the process outlined in [Appendix 2](#)
- The managing SP must update the patient journey board according to plan: green: safe for discharge, yellow: requires follow up prior to discharge, red: unsafe for discharge.
- The managing SP must update changes in the patient bedside care plan and create a safe swallowing chart if modifying the patient's diet and/or fluids as per [Appendix 3](#)
- The managing SP must collate all service events within [Allied Health System](#)

3.4 Discharges

- Decisions for patient discharge are made on a case by case basis by the treating SP.
Criteria for discharge:
 - Intervention goals achieved
 - Plateau in functional ability
 - Patient no longer benefits from intervention
 - Patient declines SP services
 - Transfer of patient to another health care service
- When patients are discharged from the AKG SP inpatient service the SP must:
 - Document the reason for discharge in the patient's medical record according to [documentation standards](#)
 - Change the journey board colour to green
 - Complete Speech Pathology Transfer summary form AKMR66.9. If the patient is being transferred to another DoH hospital, send appropriate initial contact ISOBAR electronic notes (PDF format).
 - Update NaCS as per [Appendix 4](#)

3.5 Inpatients with progressive neurological conditions

- Patients within the Armadale catchment area should be referred to Community Rehabilitation/Outpatients (CR/OP) via [e-referral](#) if appropriate.
- If residing outside the Armadale catchment area the patient should be referred to the appropriate catchment area SP services via [e-referral](#) if appropriate
- Referrals to other appropriate SP service provider (i.e. MSWA/private SP) using AKMR66.9 on discharge by the treating clinician
- If the patient is already known to CR/OP, then an email handover transfer summary will be provided to the managing clinician.

3.6 Inpatients discharged on thickened fluids

- These patients are entitled to one free bottle of fluid thickener (Precise Thick N Instant).
- The managing SP should provide a bottle of 500mls thickener and a pump to the patient from the stock on the relevant ward.

- Patients being discharged home on thickened fluids require education on thickening fluids prior to discharge, as well as information on alternative products/thickening agents available and where to purchase these, and an account established [HEN order form](#)
- The discharging clinician must complete a referral to Community Rehabilitation/Outpatients via [e-referral](#) for follow up/ongoing management

3.7 Use of Interpreter services

- The managing SP should follow [Interpreter and Translator Guideline](#) for patients of non-English speaking background

4. Legislative context

The following documents are required to give effect to this procedure [i.e. the documents included are mandatory]:

- [WA Health Consent to Treatment Policy](#)
- [East Metropolitan Health Service \(EMHS\) Consent Policy](#)
- EMHS [Patient Identification and Procedure Matching Policy](#)
- HDWA [Clinical Handover Policy 2019](#)

5. Supporting information

The following documents support the implementation of this procedure:

- [Documentation standards- clinical procedure](#)
- [Therapeutic Diet and/or Fluids procedure](#)
- SPA [Parameters of Practice: Guidelines for Delegation, Collaboration and Teamwork in Speech Pathology Practice](#)
- SPA [Scope of Practice](#)

6. Compliance, monitoring and evaluation

Compliance against this guideline will be monitored via random Speech Pathology medical record documentation audits.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.4: The health service organisation has systems for comprehensive care that:
 - a. Support clinicians to develop, document and communicate comprehensive care plans for patients' care and treatment
 - b. Provide care to patients in the setting that best meets their clinical needs
 - c. Ensure timely referral of patients with specialist healthcare needs to relevant services
 - d. Identify, at all times, the clinician with the overall accountability for a patient's care
- Standard 5 – Action 5.27: The health service organisation that admits patients overnight has systems for the preparation and distribution of food and fluids that include nutrition care plans based on current evidence and best practice

8. Policy owner (relating to these procedures)

Enquiries relating to this procedure may be directed to:

Title: A/Coordinator Speech Pathology

Department: Speech Pathology

Email: Rehana.Rees@health.wa.gov.au and Ashleigh.Vidovich@health.wa.gov.au

9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	15/11/2024	15/11/2027	Updated SCC template Clarified referral processes Addition of use of interpreters Addition of acceptance of using appropriate DMR initial contact notes as suitable transfer documentation to other DoH sites

10. Authorisation

This procedure has been authorised and issued by the Director Allied Health and Ambulatory Care

Authorised by	Danielle Kilmurray – Director Allied Health and Ambulatory Care
Authorisation date	14/11/2024
Published date	15/11/2024

Appendix 1

AKG Prioritisation tool – Speech Pathology

	Priority 1 (high)	Priority 2 (high)	Priority 3 (medium)	Priority 4 (low)	Intervention not indicated
Description	Today (this AM or PM)	Today or tomorrow	In one to two days	In 3-5 days	Intervention/SP input will NOT be provided
Response time	Same day/ASAP	Within 24 hours	Within 48 hours	As caseload allows	
	Any patient in ED where SP input will prevent admission Initial swallow assessments where patient is NBM with no PEG/NGT If discharge is planned same day and SP input is required prior to this (e.g. education, development of management plan, written handover required) Reported to be 'high risk' on current diet fluids as demonstrated by choking or coughing on meal or deteriorating condition Discharge planned for next 1-2 days and SP involvement Priority 2 patients not seen in past working day	Initial swallow assessment where patient already has PEG/NGT Initial swallow assessment for tracheotomised patients Review of tracheotomised patients where speech pathology intervention has commenced the previous day (e.g. cuff deflation regime or commenced oral intake) Attendance at family meetings or education with families when SP has a major role Priority 3 patients not seen for previous two working days	Review of swallow status where patient's medical status is variable Language/communication assessment/treatment Review of patients on modified diet/thickened fluids where upgrade is anticipated Patients requiring intensive rehabilitation as determined by the treating speech pathologist	Review of patients on modified diet/thickened fluids where upgrade is not anticipated Language/communication treatment Review of stable patients whose gains are slow	Dislikes food/poor appetite Refusing oral intake Longstanding difficulty with medications only Missing or ill-fitting dentures Pre-existing stable dysphagia and/or communication difficulties with no new changes – nursing staff to review handover information from nursing home Patients who demonstrate dementia-related feeding behaviours Longstanding non-compliance Low GCS/unable to rouse patient Med team have reported SP input not indicated Documented oesophageal dysphagia with no reported oropharyngeal symptoms

Appendix 2 - iCM – Clinical Management Handover - Speech Pathology Inpatients

Aim

Electronic handover for Speech Pathology inpatients at Armadale Health Service will occur in a standard manner. This includes handover of clinical information to ward speech pathologists, as well as electronic handover of diet/fluid recommendations to the kitchen and nursing.

Procedure

The Speech Pathology Department utilises the iCM Clinical Manager program's Health Issue (HI) feature for the handover of information regarding inpatients. The following procedures are to be completed **daily** for **all patients** that have had direct or indirect Speech Pathology Management.

Initial Patient Contact – HI Speech Pathology

Following the first contact with a patient a new HI Speech Pathology is created by selecting the HI button on the toolbar.



This opens the following screen. The steps for entering information are outlined below.

1. Type: Select <i>HI SpeechPath</i> from the drop down list.	
2. Short Name: <i>AHS SP DD/MM/YY staff initials</i> <u>Note:</u> the year must be included in the initial date.	
3. Description: See Appendix A for details of information to be included in this section. See Appendix B for a list of abbreviations used in this section.	
4. Date: select Full Date from the drop down box.	
5. Click OK to complete.	

Information to be included within the 'Description' section of the HI Speech Pathology window

When completing this field keep in mind the information should be brief and concise, however *must* provide enough detail for the incoming clinician to prioritise the patients to be seen.

Areas that are shaded are mandatory to include within entries. Other areas are at the discretion and judgement of the clinician completing the entry.

- Note – line breaks can be added to improve the clarity of entries by pressing 'ctrl+enter'.

Information	Description of what is to be included in entry.
Reason for admission.	Only if relevant and not obviously displayed elsewhere on iCM.
Previous medical history.	Only if relevant and not obviously displayed elsewhere on iCM.
History of current presentation.	Only if relevant and not obviously displayed elsewhere on iCM.
Assessment/intervention/management and results.	As appropriate and required based on patient contact and clinician judgement of what will be required for incoming clinician to prioritise patients.
Recommendations (mandatory only for patients with dysphagia)	Including: - diet and fluids - referrals - strategies - treatment - etc
Plan	The plan for ongoing <u>Speech Pathology</u> management. This needs to be specific in terms of: - timeframe - o e.g. giving a day or date that the patient should be seen, and - tasks to be completed - o e.g. dysphagia review, aphasia treatment, write discharge report Here you may also include discharge plans such as expected date and destination, or any investigations that are planned and that might inform our management (e.g. MRI, VFS, endoscopy, barium swallow).
Priority rating	In the form of: - (l) = low priority; i.e. to be seen when other cases have been attended to. - (m) = medium priority; i.e. ideally would be seen on the day indicated in the plan. - (h) = high priority; i.e. must be seen on the day indicated in the plan.

Abbreviations for use within the speech pathology iCM entries.

This list of abbreviations has been developed by the Speech Pathology Department at Armadale Health Service for use within the HI Speech Pathology iCM entries. This was identified as required due to the limited character space available within these entries.

Please note:

- Abbreviations that are acceptable for use in the medical record are also appropriate for use in iCM.
- As a general rule the below abbreviations are not appropriate for use in the medical records, unless they are also approved as per the documentation standards

Speech Terms	Abbreviations
Anterior	ant
Aphasia	da
Apraxia of Speech	<u>aos</u>
Aspiration	asp
Aspiration pneumonia	<u>AspP</u>
Bolus	<u>bo</u>
Cognition	cog
Communication	<u>comm</u>
Confusion	con
Conversation	convo
Cough	<u>cgh</u>
Cranial nerves	CN
Delayed	del
Diet and fluids	<u>d+f</u>
Dysarthria	<u>dth</u>
Dysphagia	dg
Dysphonia	<u>dph</u>
Expressive	<u>exp</u>
Hyolaryngeal excursion	HLE
Instructions	<u>inst</u>
Language	<u>lang</u>
Levels 0-7	L0-7
Mouthfuls	<u>m/fuls</u>
Oesophageal	<u>Oes</u>
Oropharyngeal	<u>OrP</u>
Penetration	pen
Receptive	rec
Residue	res
Safe swallowing strategies	SSS
Signs or symptoms	s/s
Speech	<u>sp</u>
Swallow	<u>Sw</u>
Review prior to discharge	r/v <u>or</u> d/c

General Terms	Abbreviations
and	+
Assists/assistance	A
Background	b/g
Complaining of	c/o
Declined	<u>decl</u>
Decreased	v or <u>dec</u>
Depression	<u>dpp</u>
Difficulty	<u>dif</u>
Education	<u>ed</u>
Functional	<u>fxn</u>
Impression	l: or imp
Increased	^ or <u>inc</u>
Inpatient	IP
Medical team	Drs or med
Nil further input	NFI
Nursing	<u>nsg</u>
Outpatient	OP
Plan	P:
Previous	<u>prev</u>
Prompting	P
Recommendations	R:
Refer/referral	ref
Related to	r/t
Reports	<u>rpts</u>
Secondary to	2
With	c or w
and	+

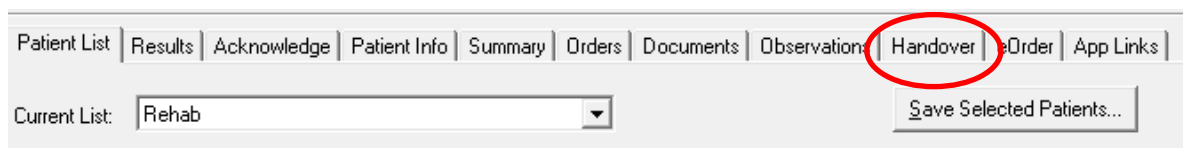
Reviews/Treatment – HI Speech Pathology

Changes should be made to the original entry throughout the patient's admission. The following changes should be made after each follow up visit with a patient.

- **Short Name:** Change the date of each review/treatment session, adjust the staff initials as necessary – *dd/mm/yy, staff initials*
- **Description:** Add any new assessment/intervention/management information, recommendation changes, and update the plan for ongoing management.
- **Note:** If the patient presents for a new admission, then a new entry needs to be created.

Updating Diet Information – Handover

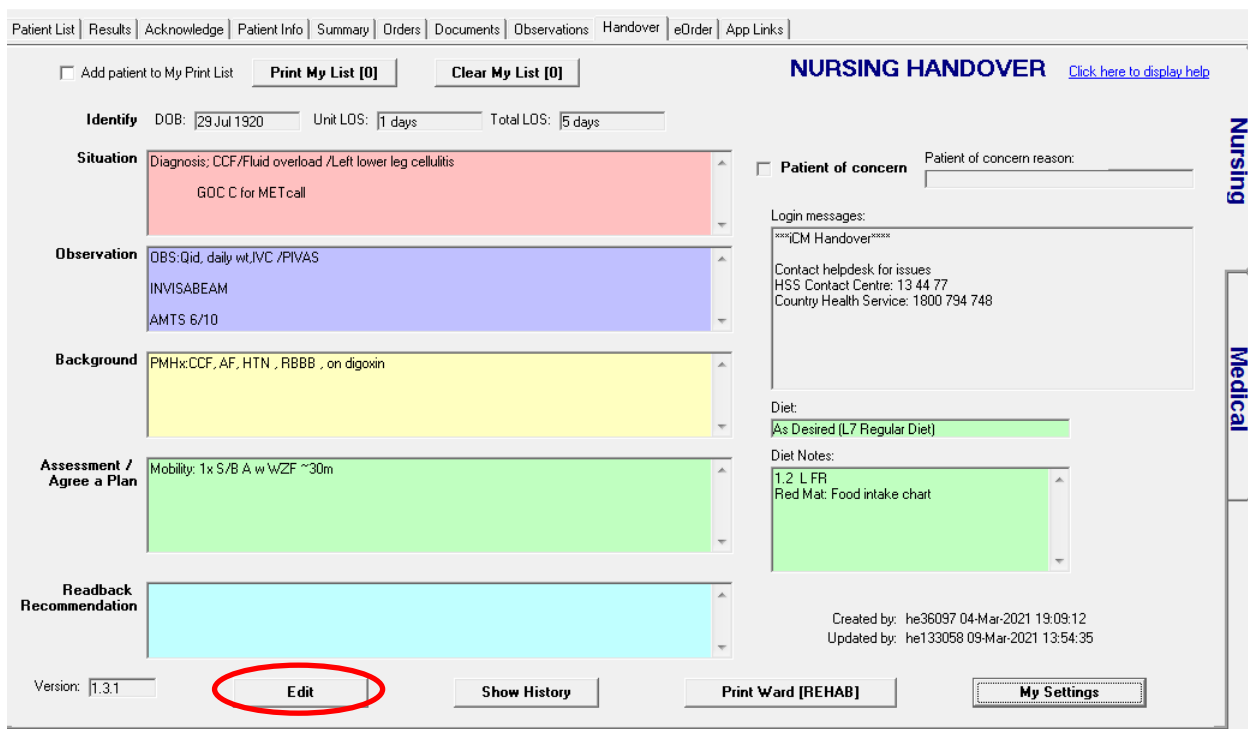
To update the patient's diet information, select the 'handover' tab.



Patient List | Results | Acknowledge | Patient Info | Summary | Orders | Documents | Observations | **Handover** | eOrder | App Links

Current List: Rehab Save Selected Patients...

This will open up a screen like this. Select 'edit'



Patient List | Results | Acknowledge | Patient Info | Summary | Orders | Documents | Observations | **Handover** | eOrder | App Links

☐ Add patient to My Print List Print My List [0] Clear My List [0] **NURSING HANDOVER** [Click here to display help](#)

Identify DOB: 29 Jul 1920 Unit LOS: 1 days Total LOS: 5 days

Situation Diagnosis: CCF/Fluid overload /Left lower leg cellulitis
GDC C for MET call

Observation OBS: Qid, daily wt, IVC /PIVAS
INVISABEAM
AMTS 6/10

Background PMHx: CCF, AF, HTN, RBBB, on digoxin

Assessment / Agree a Plan Mobility: 1x S/B A w WZF ~30m

Readback Recommendation

☐ Patient of concern Patient of concern reason:

Login messages:
****CM Handover****
Contact helpdesk for issues
HSS Contact Centre: 13 44 77
Country Health Service: 1800 794 748

Diet:
As Desired (L7 Regular Diet)

Diet Notes:
1.2 L FR
Red Mat: Food intake chart

Created by: he36097 04-Mar-2021 19:09:12
Updated by: he133058 09-Mar-2021 13:54:35

Version: 1.3.1 **Edit** Show History Print Ward [REHAB] My Settings

This opens the following screen. The steps for entering information are outlined below.

Nursing Handover - Please enter patient information suitable to match ISOBAR handover requirements

Identify DOB: 11 Dec 1944 Unit LOS: 11 days Total LOS: 11 days
Expected discharge date: 26 Feb 2021

[Click here to display ISOBAR Guidelines](#)
[Click here to display screen help](#)

Situation Diagnosis: Fall at home
CD: # R) NOF
11/2 AO cannulated screws to Femur
RDS: 26/02
NFR FOR MET

Observation OBS: TDS
Risk Mgt: Invisabeam
AMT 8/10
BD S8

Background PMHx: Parkinsons, Stroke, HTN, OP, Liver Cirrhosis, RA, Renal cell carcinoma, Chronic back pain, Dementia, vit B12 deficiency, dysphagia
ALLERGY: ? Penicillin

Assessment / Agree a Plan MOB: *Very High Falls Risk* 2x4 w Sara Steady to transfer, PWB R LL, invisabeam and to sit at front.
Continence: Continent, L4 package
Wounds: as per WCMP, TDS Betadine wipes to scrotum tear

Readback recommendation Confirm shared understanding
SOCIAL: Lives with wife,

Patient of Concern Reason:

Print results on Handover Report Filter by: [None]

PerformedDtm	ItemName	ItemValue	UOM

Patient information - Check box to print on handover report

CI	TypeCode	Text
☐	ADDRESS	389 BICKLEY ROAD KENWICK
☐	FINANCIAL CLASS	PRIVATE INSURED
☐	GP	PAUL AH-HUI TAN (08 93624932)
☐	MEDICARE_NO	6031459351-1
☐	MRSA UPDATE	UNK
☐	VISIT REASON	Post screws fixation to R) Hip, 2nd to #R)NOF post fall

Diet: Level 6 Soft and Bite Sized Diet

Diet Notes: Level 2 Thickened fluids
Red Mat: Full assist with all meals/needs encouragement to eat. Reduced dexterity due to arthritis.

Created by: he37809 26-Feb-2021 12:42:0
Updated by: he20351 05-Mar-2021 09:44:1

Save & Close **Cancel**

Select save & close to exit

Enter the diet name

Enter any relevant diet notes/fluid levels etc here

Nursing Handover

Nursing staff use the 'Handover' tab separately to ensure ISOBAR requirements are met.

Every time that a patients' diet is changed, please ensure it is also changed on the Handover tab. See above pictures for details.

- Select patient to change
- Select handover tab
- Click 'Edit' as shown above
- Click on green diet down box and choose diet/fluid type from menu
- Save & close

Repeat Admissions

If a patient seen previously by the Speech Pathology Department is readmitted, the following steps should be taken:

- Make previous Speech Pathology HIs which are no longer relevant "Inactive".
- Create a new Speech Pathology HI for the current admission.
- Note within the description section of the new HI that the patient is known to Speech Pathology.

To view inactive HIs:

- Select the “Summary” tab.
- Click the  icon next to the “Active Health Issues” heading.
- You will be shown a list of health issues with inactive issues appearing *italicised*.

To preview or print out a report

1. Enter Clinical Manager
2. Open relevant ward list by clicking:
Current list – select ward from drop down list
3. Click on the printer symbol on the tool bar
4. Select Report Category: Patient List
5. Select Customised List –DIET Report, then select Options
6. Holding down the control button on your keyboard, choose Admiss 10 and HI SpeechPath
7. Press preview
8. If happy with report – print out

Appendix 3 – Example of safe swallowing chart

Safe Swallowing Chart			
Name: Click here to enter text.		Date: 26/09/2024	
Ward: Choose an item.			
			
Choose an item.	Choose an item.		
	Choose an item.		
	Choose an item. Choose an item.		
	Choose an item. Choose an item.		Choose an item.
Standard safe swallow strategies:			
	Must be fully awake & alert		Minimise distractions during mealtime
	Must be sitting fully upright		Stay upright for at least 20 mins after meals

For this patient:			
	Make sure each mouthful is swallowed before giving the next		Remove any pocketed food from left/right cheek after meals
	Alternate food & drink during meals		Ensure small mouthfuls
	Regular oral care		Full oral care after meals
	May have small sips of thin drinks with supervision		Patient allowed Level 7 Regular snacks with supervision

Rehabilitation Strategies			
	Double swallows for each mouthful		Keep chin down when swallowing
	Effortful swallow ("Swallow hard, with all your muscles")		Cough, clear & swallow every 4-5 mouthfuls
	Place food in the right side of the mouth		Place food in the left side of the mouth

Stop feeding if coughing or tired.	
	If necessary, please refer to speech pathology or administer the Dysphagia Screening Tool (DST).
Speech Pathologist: Click here to enter text.	Contact Number: Click here to enter text.

Appendix 4 - GUIDELINES FOR SPEECH PATHOLOGY DOCUMENTATION IN NACS

Criteria for documentation	Example
Pt is being discharged on modified diet and fluids	SP (<i>initials</i> , pager#): "Pt presented with oro-pharyngeal dysphagia secondary to XXXX (<i>diagnosis e.g. history of PD/COPD/stroke</i>). Requires XXXX diet and XXXX fluids for safe swallowing"
Pt is at risk of aspiration/needs to be monitored for indicators of aspiration	SP (<i>initials</i> , pager#): "Pt is at risk of aspiration with XXXX and would benefit from ongoing monitoring of his/her chest status and for signs of aspiration."
Pt requires follow-up with another specialty e.g. ENT, Gastro, Neuro	SP (<i>initials</i> , pager#): "Pt presented with XXXX and <u>may</u> benefit from consultation/review by XXXX"
Pt requires ongoing SP input i.e. via out-pts, CR, RITH	SP (<i>initials</i> , pager#): "Pt presented with XXXX and required XXX therapy as an in-patient. Pt would benefit from ongoing SP input as an out-patient through XXXX(<i>service</i>)"
New communication impairment	SP (<i>initials</i> , pager#): "Pt presented with <i>mild/moderate/severe aphasia/dysarthria/dyspraxia/dysphonia</i> secondary to XXXX (<i>diagnosis e.g. stroke</i>). Requires XXXX (e.g. ongoing SP input as out-pt; assistive and augmentative communication device, iPad/communication board/Provox to communicate).
Extensive input as an inpatient where follow-up is not required	SP (<i>initials</i> , pager#): Pt had extensive input as an inpatient due to acute dysphagia/communication impairment. Pt has returned to baseline and no further SP input is required.